

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°136/MVE-BDBV/09/27/2026

Reporting date: September 27, 2026

Publication date: September 28, 2026

**HIGHLIGHTS** (last 24 hours)

A total of 49 new confirmed cases of Ebola Virus Disease (EVD) were reported, including 18 community deaths. The confirmed cases were distributed across the provinces of Ituri (27 cases), North Kivu (18 cases), and Haut-Uélé (4 cases). In addition, 5 deaths were recorded in Ebola Treatment Centers (ETCs).

A total of 270 new samples were analyzed, 49 of these samples were found to be positive for the Bundibugyo virus, representing 18.1% positivity.

No new health zones have been affected in the last 24 hours.

ACCUMULATIVE



CASE

8,116



ACCUMULATIVE

DEATH
CONFIRMED

3,924



LETHALITY

48.3%

PATIENTS IN

ISOLATION/CTE



817

ACCUMULATIVE



HEALED

2088

TRACKING RATES



CONTACTS

(Of the Day)

75.9%



Affected provinces and affected health zones



7 Provinces affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo, Bas-Uélé and
South Ubangi



63 out of 167
health zones affected
(37.7%)

Summary of key points from the last 24 hours

On September 27, forty-nine (49) new confirmed cases of Ebola Virus Disease (EVD), including eighteen (18) community deaths, were recorded. The confirmed cases came from the provinces of Ituri (27 cases), North Kivu (18 cases) and Haut-Uélé (4 cases).

In addition, 5 deaths were recorded in the CTEs (4 in Ituri and 1 in North Kivu), bringing the total number of confirmed deaths to 23 at the end of the day.

As of September 27, 2026, the DRC had recorded 8,116 confirmed cases and 3,924 deaths (case fatality rate: 48.3 %). Ituri remained the epicenter of the epidemic, accounting for 76.2% of all confirmed cases. In total, 63 out of 167 health zones (37.7%), spread across seven provinces, had been affected since the start of the outbreak.

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 27, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	27	6182	2,848	46.1%	28/36 (77.8%)
North Kivu	18	1541	914	59.3%	16/34 (47.1%)
Haut-Uélé	4	335	140	41.8%	7/13 (53.8%)
Tshopo	0	43	16	37.2%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	10	4	40.0%	3/11 (27.3%)
South Ubangi	0	2	1	50.0%	1/16 (6.3%)
Total	49	8,116	3,924	48.3%	63/167 (37.7%)

Table 2. Distribution of confirmed cases and deaths by province and zone as of September 27, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	6182	2848	46.1%	27	10	4	14
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	202	46	22.8%	1	0	0	0
Boga	2	2	100.0%				0
Bunia	1705	550	32.3%	8	5	2	7
Damascus	40	9	22.5%				0
Drodro	15	7	46.7%				0
Fataki	82	31	37.8%				0
Gethy	7	3	42.9%				0
Kambala	2	0	0.0%				0
Kilo	35	12	34.3%				0
Komanda	167	101	60.5%	1	1	0	1
Lita	283	141	49.8%	0	0	1	1
Logo	13	5	38.5%				0
Lolwa	33	8	24.2%				0
Mahagi	4	1	25.0%				0
Mambasa	29	11	37.9%	2	2	0	2
Mandima	120	67	55.8%	2	2	0	2
Mangala	346	153	44.2%				0
Mongbwalu	698	328	47.0%	1	0	0	0
Nia-Nia	293	159	54.3%	9	0	0	0
Nizi	772	278	36.0%	1	0	1	1
Nyankunde	128	37	28.9%	1	0	0	0
Rimba	12	4	33.3%				0

Rwampara	1079	363	33.6%	1	0	0	0
Tchomia	76	25	32.9%				0
To ventilate *	N / A	493	N / A				0
North Kivu	1541	914	59.3%	18	7	1	8
Blessed	337	222	65.9%	8	2	0	2
Good	18	7	38.9%				0
Butembo	257	166	64.6%	1	1	0	1
Goma	1	0	0.0%				0
Kalunguta	53	18	34.0%	2	2	0	2
Katwa	559	355	63.5%	4	1	1	2
Kayna	1	0	0.0%				0
Kyondo	33	20	60.6%				0
Lubero	21	11	52.4%				0
Mabalako	31	11	35.5%				0
Manguredjipa	13	5	38.5%				0
Masereka	28	9	32.1%				0
Musienene	122	66	54.1%	1	1	0	1
Mutwanga	12	2	16.7%				0
Oicha	26	14	53.8%	2	0	0	0
Vuhovi	29	8	27.6%				0
Haut-Uélé	335	140	41.8%	4	1	0	1
Boma Mangbetu	39	17	43.6%				0
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	101	43	42.6%	1	0	0	0
Pawa	80	37	46.3%	3	1	0	1
Rungu	2	1	50.0%				0
Wamba	109	40	36.7%				0
Tshopo	43	16	37.2%	0	0	0	0
Bafwasende	7	1	14.3%				0
Kabondo	5	1	20.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	23	10	43.5%				0
Mangobo	5	3	60.0%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	10	4	40.0%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	6	1	16.7%				0
Ganga	3	2	66.7%				0
South Ubangi	2	1	50.0%	0	0	0	0
Bulu	2	1	50.0%				0
Total	8116	3924	48.3%	49	18	5	23

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified by province for September 27, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	7	4	11	7	4	0	0	11	7
Bas Uélé	114	1	115	8	1	106	0	9	0
Ituri	782	72	854	88	72	690	0	160	62
North Kivu	761		761	77	21	630	0	98	46
South Ubangi	14	0	14	4	0	10	0	4	0
Tshopo	57	0	57	1	0	56	0	1	0
Total General	1,735	77	1,812	185	98	1,496	0	283	115

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

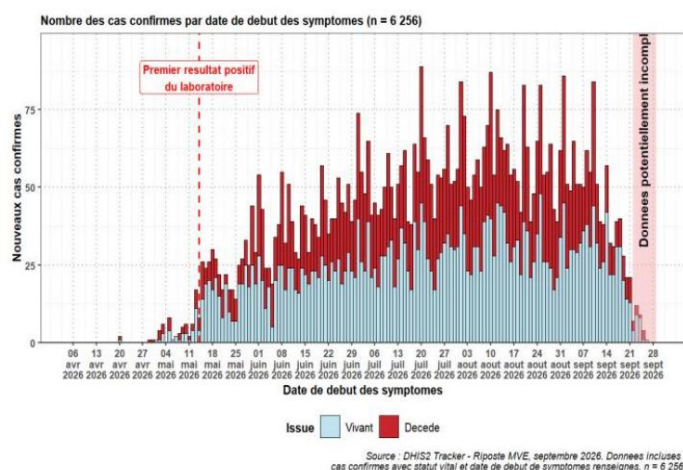
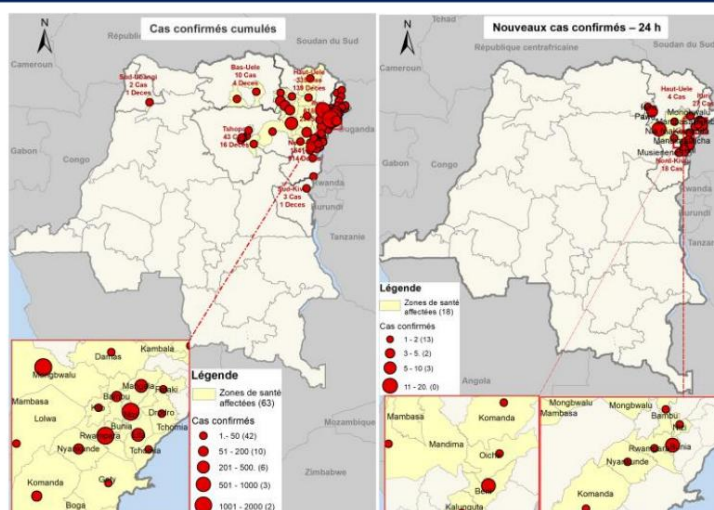


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

In total, 1,812 alerts were notified during the day of September 27, 2026.

1,779 (98.2%) were verified. Of the verified alerts, 283 (15.9%) were validated as suspected cases. All validated alerts were investigated (100%).

In total, 115 suspected cases were transferred to CT/CTE for appropriate management (Table 3).

The follow-up rate as of September 27, 2026, was 75.9%. This rate was 88.8% (87/98) in Sud-Ubangi, 86.6% (123/142) in Bas-Uélé, and 79.1% in [unclear].

% (778/984) in Haut-Uélé, 78.0% (592/763) in Tshopo, 75.7% in Nord-Kivu and 73.2% in Ituri.

For comparison, the contact tracing rate was 74.7% on September 26, 2026, compared to [validated value] on September 27, 2026. This proportion was 88.8% (87/98) in South Ubangi, 86.6% (123/142) in Bas-Uélé, 79.1% (778/984) in Haut Uélé, 75.7% (11,819/15,611) in North Kivu, 73.2% (9,829/13,424) in Ituri and 71.1% (551/775) in Tshopo.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

For the day of September 27, 2026, the strategic PoE/PoC were **100%** functional in the provinces of Ituri, North Kivu and Tshopo.

In total, **247,045 travellers** were registered at the PoE/PoC. Among them, **97.5%** were screened, **97.7%** washed their hands and **99.0%** benefited from awareness sessions.

One (**1**) alert was notified to the PoE/PoC during the day. This alert, recorded in Ituri, was validated as a suspected case, investigated within the required timeframe, and transferred to a care facility, representing a transfer rate of **100%**.

Table 4. Status of key PoE/PoC indicators by province, as of September 27, 2026

Indicators	Ituri	North Kivu	Tshopo	Overall
Report completeness	93.3% (59/60)	95.2% (20/21)	100% (18/18)	95.0% (94/99)
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	132,860	93,183	218,002	247,045
% of travelers screened	99.0%	97.7%	98.8%	97.5%
% of travelers who washed their hands	97.4%	97.7%	98.9%	97.7%
% of travelers made aware	99.5%	97.7%	100.0%	99.0%
Number of alerts notified	1	0	0	1
Number of validated live alerts (suspected cases)	1	0	0	1
Number of bodies intercepted today % of	0	0	0	0
suspected cases transferred to CTE/CT	100.0%	N / A	N / A	100%
% of lifeless bodies swabbed/secured	0.0%	N / A	N / A	0.0%
% of suspected cases investigated within 4 hours	100.0%	N / A	N / A	100.0%

2.2.2. Challenges

Despite 100% functionality of **strategic PoE/PoC**, coverage of screening (**97.5 %**) and handwashing (**97.7 %**) remains below the operational target of 100%.

Moreover, the completeness of the reports remains insufficient (**95.0%; 94/99**), particularly in Ituri (**93.3%; 59/60**) and North Kivu (**95.2%; 20/21**).

The only validated PoE/PoC alert was reported in Ituri, highlighting the need to maintain a high level of vigilance in areas of active transmission.

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 27 new positive results (17 living and 10 dead) out of 137 samples received and analyzed (positivity: 19.7%).**
- **North Kivu: 18 new positive results (18 living and 7 dead) out of 109 samples received and analyzed (positivity: 16.5%).**

- **Haut-Uélé: 4 new positive results out of 21 samples received and analyzed (positivity: 19.0%).**
- **Tshopo: 1 sample received and analyzed (1 live sample), it tested negative**
- **Bas Uélé: 2 samples received and analyzed (1 living and 1 deceased), all returned negatives**

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 23 out of 37 rings were opened (62.2%), 53 out of 65 households were decontaminated (81.5%), and 11 out of 23 health facilities were decontaminated (47.8%). A total of 87 EDS alerts were reported, 60 EDSs were carried out, and 59 bodies were swabbed.
- **North Kivu :** 30 out of 36 rings were opened (83.3%), 6 out of 21 households were decontaminated (28.6%), and 26 out of 28 health establishments were decontaminated (92.9%). In total, 185 health facilities were monitored, 488 service providers were briefed, and 39 health impact assessments were carried out.
- **Haut-Uélé :** IPC activities continued in the affected areas with monitoring of health facilities, staff awareness-raising and decontamination activities around reported cases.
- **Tshopo :** 15 health establishments were monitored in 6 health zones and 164 service providers were briefed. Cumulatively, 25 out of 28 households (89.3%) and 33 out of 33 health establishments (100%) that had hosted a confirmed case were decontaminated within 48 hours.

2.4.2. Challenges

In Ituri, ring opening coverage (62.2%) and decontamination of health facilities (47.8%) remain low, while 19 bodies have not been swabbed. **In North Kivu**, household decontamination remains limited (28.6%; 6/21) and the timeliness/completeness

The PCI reporting rate is 62.5% (10/16 health zones). **In Haut-Uélé**, the low level of PCI data reporting and the lack of suitable infrastructure in some areas continue to affect the implementation of activities.

In Tshopo, only 2.0% of social and solidarity economy (SSE) communities (2/99) achieved a PCI score above 80%, while household coverage of PCI/WASH kits remains limited to 40.0% (20/50).

2.5. Holistic care

2.5.1. Main actions

- **In Ituri**, 45 new admissions were recorded, compared to 57 discharges, including 7 recoveries. A total of 368 patients remained in isolation at the end of the day, including 162 confirmed cases and 218 suspected cases. The bed occupancy rate was 36.3%.
- **In North Kivu**, 82 new admissions were recorded against 65 discharges, including 6 recoveries. A total of 345 patients remained hospitalized, including 102 confirmed cases and 243 suspected cases. The bed occupancy rate was 63.0%.
- **In Haut-Uélé**, 14 new admissions were recorded against 6 discharges, including 3 recoveries. A total of 68 patients remained in isolation, including 30 confirmed cases and 34 suspected cases. The bed occupancy rate was 49.3%.
- **In Tshopo**, no new admissions were recorded. Four discharges were reported, including 2 recoveries and 2 non-cases. Thirteen patients remained in isolation at the end of the day, including 4 confirmed cases and 6 suspected cases.
- **In Bas-Uélé**, 5 new admissions were recorded. Nine patients remained in isolation at the end of the day, including 5 suspected cases.

In total, **146 new admissions** and **132 discharges**, including **18 recoveries**, were recorded. Thus, **803 patients** remained in isolation at the end of the day.

2.5.2. Challenges

- **In Ituri**, the limited capacity of some CTE/CT facilities and the inadequacy of care food-related issues remain a concern.
- **In North Kivu**, a significant proportion of patients remain suspected cases (243/345; 70.4%), increasing the pressure on isolation capacities.

- **In Haut-Uélé**, the increase in admissions (+14) compared to discharges (+6) continues to increase the burden on care structures.
- **In Bas-Uélé**, the lack of a standardized CTE and ambulance for the transfer of patients remains a major operational challenge.
- **In Tshopo**, the continued presence of confirmed cases in isolation necessitates the maintenance of infection prevention and control measures.

2.6. Vaccination

2.6.1. Main actions

In Ituri, following harmonization of data from September 26, 158 people were vaccinated with MSF support, including 54 women and 104 men, with no reported adverse events following immunization (AEFIs). The cumulative total reached 536 people vaccinated, including 217 women and 319 men. Vaccination activities and research on the efficacy of the ERVEBO vaccine against BBDV-EV are ongoing.

- **In Bas-Uélé**, 500 doses of the ERVEBO vaccine were received and a generator was installed to power the ultra-low temperature cold chain. The provincial total has reached 1,000 people vaccinated out of a target of 13,202, representing 7.6%. Vaccination continues in Buta, and preparations are underway in Aketi.

- **In Tshopo**, the latest report, dated September 26, indicates that 4,953 people vaccinated against PPL/TPL have reached a target of 11,703, representing 42.3%. The stockpile includes 47 thawed doses and no frozen doses at the provincial depot.

2.6.2. Challenges

In Tshopo province, there is a shortage of frozen ERVEBO vaccine and no doses are available in the health zones of Kabondo, Tshopo, and Bafwasende, in a context of high demand. Vaccination coverage remains low in Lubunga, with 274 out of 1,646 people vaccinated (16.6%), and in Mangobo, with 540 out of 2,305 (23.4%).

In Bas-Uélé, the ultra-low temperature cold chain has been effectively put into operation and vaccination coverage has been extended.

In Ituri, the target for PPL/TPL remains to be redefined; no reliable coverage rate is calculated on this basis in the presentation.

2.7. Continuity of Care

2.7.1. Main Actions • In Tshopo,

the activities presented for September 26 included the evaluation of the ECC/Makiso Medical Center, the development of recovery plans for the SIMAMA CRP and the VITA NOVA CM, the evaluation of the pillar's weekly work plan, and the monitoring of PPLs at the CTM and CTE. All 20 staff members at the visited facility have been vaccinated, including 13 PPLs. Effective use of the partogram and the presence of a functional incinerator were noted.

2.7.2. Challenges

In Tshopo, efforts are focused on strengthening understanding of the use of infrared thermometers, infection prevention and control (IPC) measures, and communication regarding the continuity of essential care. The temporary closure of the DHKis ESS (Emergency Health Service) is among the priority actions proposed in response to the increasing number of confirmed and suspected cases.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In Ituri**, 15,274 home visits reached 40,679 people; 665 educational talks brought together 14,869 people and 228 mass awareness campaigns reached 6,360 people.

The teams reported 43 public service announcements and 9 radio broadcasts. A total of 498 community alerts, including 78 deaths, were reported; 212 out of 261 feedback requests (81.2%) and 12 out of 19 incidents (63.2%) were resolved within 48 hours. CREC assisted with the discharge and reintegration of 121 recoveries from the Mongbwalu Ebola Treatment Center.

- **In North Kivu**, 7,686 home visits reached 25,216 people, 148 educational talks brought together 4,065 people, and 41 mass communication sessions reached 8,202 people. Two briefings involved 43 participants. Four out of five incidents (80.0%)

The cases were resolved in Biena, Lubero and Mabalako. The CREC supported the listing of 119 contacts in Mabalako and the publicizing of the release of recovered patients in Manguredjipa and Kyondo.

- **In Haut-Uélé**, 523 home visits reached 1,459 people, 45 educational talks brought together 381 people, and 4 mass activities reached 4,795 people. In Watsa, 60 stakeholders, including 20 leaders and 40 heads of healthcare facilities, were briefed on surveillance, CREC (Community-Based Risk Reduction), and IPC (Infection Prevention and Control) with the support of Caritas Congo.
- **In Tshopo**, the teams raised awareness among 34 religious denominations, conducted 6 educational talks, and continued visits to 244 households. The report indicates that 26,449 people were reached, including 11,563 men and 14,886 women, and that 4 interactive broadcasts were also conducted. The 24 community alerts were addressed, the 2 feedback issues resolved, and the 2 refusals were lifted.
- **In Bas-Uélé**, activities have been intensified around the confirmed case in Viadana. Teams report raising awareness among 1,833 worshippers in Ganga and Viadana, as well as 28 registered nurses in Ganga, regarding the reporting of alerts. The Viadana report indicates 8 home visits reaching 47 people, mass awareness campaigns targeting 652 people, and 5 community alerts addressed.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- **In North Kivu**, 16 out of 17 new confirmed cases (94.1%) benefited from SMSPS interventions in 5 of the 6 health zones that reported cases. The teams supported 25 households with separated and orphaned children, monitored 2 children in daycare and 26 in the community, and reintegrated 2 recovered children.
- **In Ituri**, 21 new confirmed cases out of 57 (36.8%), 89 confirmed cases under follow-up out of 151 (58.9%), 31 new suspected cases out of 45 (68.9%), and 88 suspected cases under follow-up out of 138 (63.8%) benefited from SMSPS interventions. The teams supported 420 caregivers and 96 people living with HIV, monitored 23 out of 28 recoveries in the community (82.1%), and announced 60 results, including 21 positive ones. Seven recoveries were reintegrated into society, and 15 children were monitored in daycare centers.
- **In Tshopo**, the teams carried out psychoeducation for 101 people, announced 7 negative results and reintegrated one recovered person.

2.9.2. Challenges

- **In Ituri**, rapidly strengthen the capacities of psychologists and psychosocial workers for interventions related to cases in Mandima.
- **In Tshopo**, train the psychosocial agents of Wanie-Rukula and Bafwasende, strengthen the means of communication and movement of the pillar, and provide material support to the family of the healed person of Bafwandambo.

2.10. Logistics

2.10.1. Main actions

- **In North Kivu**, delivery of medicines and PCI inputs from the UG-PDSS donation and the DPS buffer stock to the Kyondo health zone; packing of a batch for Musienene and continuation of the construction of the CTE at the Musienene General Referral Hospital.
- **In Tshopo**, monitoring of the construction of the Bafwasende CTE, provision of inputs for the health areas of Bafwasende, provision of 3 mattresses, 2 pairs of loincloths and 2 pairs of sheets, and support for the mobility of pillars.
- **In Bas-Uélé**, support for the deployment of surveillance teams in Ganga and Viadana, continuation of the construction of the transit center of the HGR in Buta and rehabilitation work on the premises of the DPS with the support of the WHO.

2.11. Security

2.11.1. Main actions

- In North Kivu, the Risk Mitigation and Security pillar reports the support of EDS teams for 7 burials of confirmed cases at the cemeteries of Bundji, Vulindi and Kitovo, with a declared security coverage of 100%.

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

In Ituri, 48 community actors, including 16 women and 32 men, were briefed in Mangala and signed the code of good conduct; 41 people were sensitized in Bogoro.

In Haut-Uélé, 104 people participated in awareness sessions: 9 in Tely on the roles of community complaint management mechanisms, 45 members of the Justice and Peace Commission in Telalingbi, and 50 people at the CASA SONIA boarding school in Wamba. The sessions focused on the PSEA (Public Service for the Elderly) and reporting and referral mechanisms.

2.12.2. Challenges

In Haut-Uélé, there is insufficient coverage of affected areas, a lack of awareness materials and codes of good conduct, as well as means of transport, computer equipment and office supplies.

KEY MESSAGES

- The epidemiological situation remains worrying. In Ituri, 27 new confirmed cases were reported on September 27 compared to 57 the day before, a decrease of 52.6%, without sufficient evidence of a slowdown in transmission.
- 153/160 suspects were investigated within 24 hours (95.6%). Contact tracing remains insufficient: 71.6% in Ituri, 74.1% in North Kivu, 77.7% in Tshopo, and 68.1% in Bas-Uélé. Locating contacts who have never been seen or have been lost to follow-up remains a priority.
- Strengthen surveillance, care, infection prevention and control (IPC), logistics, and community engagement, prioritizing increased capacity at Ebola Treatment Centers (ETCs)/Treatment Centers (TCs) in North Kivu, rapid transfer of suspected cases, and isolation of confirmed cases. In Mangobo, two confirmed cases who escaped from the ETC remain reported at home.
- Strengthen preparedness and response to Ebola Virus Disease (EVD) in the river corridor and surrounding areas, particularly in South Ubangi, through active case finding, contact tracing, and ensuring the availability of supplies. Continue cross-border surveillance with South Sudan.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



EBOLA PRESIDENTIAL FORCE TASK 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System

Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: diedonne.mwamba@insp.cd

Incident Manager

Professor Dr. AHUKA MUNDEKE Steve

Email: steve.ahuka@unikin.ac.cd

Operations Manager

Dr. KOMBE NGWAMA John

Editorial Team

Professor Dr. MULANGU KATULUMBA Félix-Médard

Dr. Jules Katusi Biselenge

Mrs. MAYELE AFUA Marie

Dr. DIUR MANANASHI Floryan

Mr. KAMUINA KEBELA John

Contact :

www.insp.cd

contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd